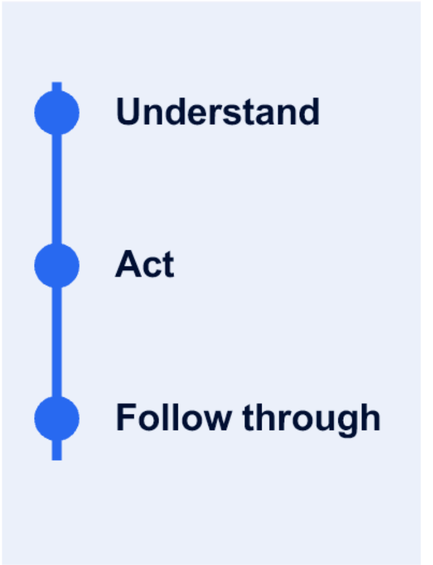
HiLabs™

APM CASELET 2026–27

CareFlow

A case management workbench

More time for clinical judgment.
Clearer action for every member.



Akshath Murali

ME23B010

HiLabs

One clinician coordinates many ongoing cases

Primary user: the health-plan case manager. Patients are members, not app operators.

Maria, RN case manager

20 years of ICU experience
48 active cases
6 discharge alerts since Friday

What she does in each case

01 **Identify & assign**

02 **Review & contact**

03 **Assess & plan**

04 **Coordinate & verify**

05 **Document & transition**

The case stays open for weeks or months. Each call, task and source update belongs to that ongoing plan.

Workload and experience: brief scenario inputs. Framework: ncqa.org/programs/health-plans/case-management-cm/

HiLabs

Five problems consume time and delay follow-up

Priority is a research-informed product judgment, not an observed effect size.

Problem	Score / 10	What CareFlow changes
Fragmented or conflicting patient picture	9.55	Source-aware summary and timeline
Unclear daily priorities	8.90	Explained queue and next action
Repetitive documentation	8.75	Reusable facts and reviewed note drafts
Lost tasks and referral follow-through	8.75	Owner, review date and outcome evidence
Ineffective outreach	8.55	Permission, route and contact history

Where the day goes



Documentation	120 min
Information search	90 min
Failed outreach	60 min
Coordination	60 min
Conversations	90 min
Other duties / breaks	60 min

480 min · illustrative, not measured

Evidence anchor

In one historical health-plan study (80 respondents), 68% rated duplicate entry and 49% rated multiple logins as major barriers.

Score weights: patient impact 35%, time burden 25%, reach 20%, MVP addressability 20%. S2: pmc.ncbi.nlm.nih.gov/articles/PMC6557451/ (survey 2012–13).

HiLabs

Clinical data needs provenance and a useful decision

Freshness targets below are proposed service levels, subject to source availability.

Dataset	Source / desired freshness	Decision enabled	Quality issue and handling	Scope
Eligibility	Plan enrollment; daily goal	Member, PCP and benefit context	Show effective date and last verification	MVP
Cases	Workbench; immediate	Owner, state and reason for care	Warn on duplicate active enrollment	MVP
Hospital alerts	Hospital / HIE; minutes–hours goal	Recognize admission and discharge	Deduplicate events; separate event and receipt time	MVP
Clinical documents	Provider / HIE; latest available	Understand instructions and clinical context	Keep original versions; flag conflicting or missing facts	MVP
Pharmacy fills	PBM; daily goal	Verify possible supply or access gaps	Show coverage; a fill is not proof of ingestion	MVP
Claims	Insurer; as received, often lagged	Historical utilization and diagnoses	No claim does not prove no care	MVP
Appointments	Provider / manual; daily goal	Arrange and verify follow-up	Separate scheduled, attended and unknown	MVP
Contact preferences	Plan + member; each contact	Choose route, language and time	Verify changes; preserve invalid-route history	MVP
Outreach history	Workbench; immediate	Learn from previous attempts	Controlled outcomes; explicit next review	MVP

Example: discharge + medication list + fills + appointment status = verify medicine access and follow-up. Missing fill creates a question, not a diagnosis.

HiLabs

Operational data completes the care loop

Standardization is upstream. CareFlow decides what the clinician sees and does.

Dataset	Source / desired freshness	Decision enabled	Quality issue and handling	Scope
Assessment / social needs	Member / caregiver; on change	Build feasible patient goals	Unknown is not none; show assessment age	MVP
Tasks and referrals	Workbench + services; as supplied	Track help through actual receipt	Owner and deadline; sent differs from received	MVP
Care plans / notes	Clinician; each interaction	Document goals and progress once	Review reused facts; sign and version changes	MVP
Consent / permissions	Plan + confirmation; before action	Check appropriate contact and disclosure	Caregiver scope, expiry and access checks	MVP
Service directory / benefits	Plan + partners; verify before use	Find eligible and available support	Show last check; do not assume capacity	Curated MVP
Risk / care-gap signals	Existing plan analytics; per run	Support case finding and review	Explain inputs; avoid cost-only ranking	MVP inputs
Staff / audit events	Identity + workbench; immediate	Assignment and accountability	Role checks and retained change history	Basic MVP
Authorization status	Insurer UM; daily when integrated	Find and escalate coverage barriers	Manual status/link first; no automatic denial	Later
Home-monitoring data	Device / provider; by agreement	Support clinically governed monitoring	Validate units, gaps and alarm policy	Later

Example: housing need + referral stage + transport need = verify availability and arrange feasible follow-up. Full field inventory: Excel annex.

HiLabs

Three patients expose different information failures

Demo clock: Monday, 9 November 2026 at 08:00 US Eastern. All records are fictional.

Patient	Care need	Planted imperfection	What Maria sees and does at 8 am
SYN-001 Avery Vale, 68	Recent heart-failure discharge	20 mg clinic list vs 40 mg discharge list; duplicate alert	Review the conflict. Call this morning. Verify access and route dose reconciliation to the treating team. Confirm Nov 12 visit and transport.
SYN-002 Jordan Reed, 54	Diabetes with possible refill gap	Network-only fill history; old note says adherent	Prepare now, call after 17:30. Confirm actual supply and affordability before assuming nonadherence. Coordinate pharmacy and follow-up.
SYN-003 Riley Moss, 39	Repeated ED use and failed outreach	Disconnected phone; missing ED details; stale social assessment	Review the Nov 7 event. Seek a permitted alternate route through the care team. Arrange Spanish outreach. Clinical status remains unknown.

SOURCE SIGNAL → UNCERTAINTY → VERIFICATION → OWNED ACTION

Original patients, not the brief's example. Stable unique member IDs across datasets. Imperfections stress decisions, not identity-matching infrastructure.

HiLabs

Social needs and caregiver capacity reshape the plan

The annex contains source records, source dates, quality issues and an action line for every member.

Patient	Care need and imperfection	What Maria sees and does at 8 am
SYN-004 Morgan Brook, 31	Behavioral health and housing instability. Referral sent but no service received; old address.	Prepare a 14:00 call. Verify housing referral and availability. Respect neutral voicemail and arrange transport for the Nov 10 visit.
SYN-005 Casey Finch, 81	Recent discharge and caregiver strain. Home-health start unconfirmed; medication-status conflict.	Prepare a callback after 16:00 to the authorized daughter. Verify home-health start and transport. Route conflicting instructions to the treating team.

13 linked datasets per patient

Identity + case

Member · case · permissions

Clinical context

ADT · records · fills · claims

Action + follow-up

Visits · outreach · tasks

Plan + signals

Assessment · notes · risk

Unavailable records remain explicit. A referral can be accepted while the patient's need is still unmet. No real PHI, providers, calls or bookings.

HiLabs

The daily queue makes urgency explainable

Above the fold: member, reason, priority, next action, due date and preferred contact time.

CareFlow

Workspace | Avery Vale | Riley Moss | +

- Overview
- **Work queue**
- Cases
- Tasks
- Insights

My work

4 priority reviews 1 overdue action

Member / tier	Why now	Next step
Morgan / P1	Housing action overdue	Verify referral at 14:00
Avery / P1	Discharge; medication conflict	Prepare and call this morning
Casey / P1	Home-health start unknown	Prepare; call after 16:00
Riley / P1	ED event; disconnected phone	Verify alternate route
Jordan / P2	Possible access or refill gap	Call after 17:30

WALKTHROUGH 01

Avery’s transition

01 Open the discharge case from the queue.

02 Read why it ranks P1 and which facts need review.

03 Prepare the morning call and follow-up actions.

Rule-based ordering: tier, overdue work, due time, then event recency. Show evidence and allow reasoned overrides. No cost-only ranking or new predictive model.

HiLabs

The patient view preserves conflicting evidence

Pinned identity stays visible while source evidence, timeline and care actions share the workspace.

CareFlow

Overview
Work queue
Cases
Tasks
Insights

Avery Vale **SYN-001** **CASE-SYN-001** **Maria RN**

CONFLICTING

Medication instructions need confirmation

Nov 6 Discharge list: furosemide 40 mg daily

Oct 20 Clinic list: furosemide 20 mg daily

Next action

Confirm access. Ask the treating team to reconcile the dose.
Do not silently choose one source or change a prescription.

WALKTHROUGH 01

Evidence before action

01 Compare the original statements and dates.

02 Distinguish source conflict from current clinical truth.

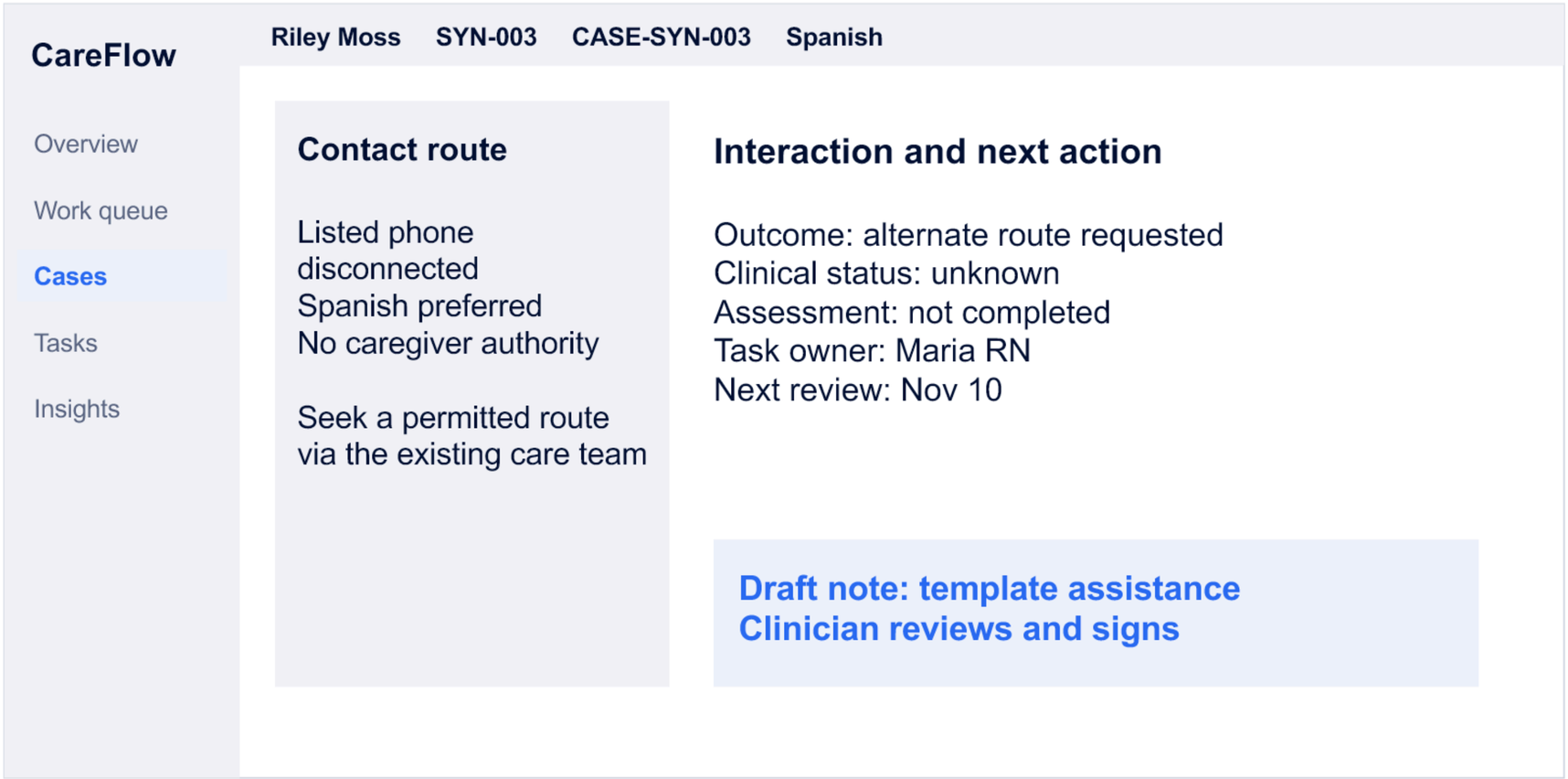
03 Create an owned reconciliation task and verify follow-up.

Every insight links to supporting records. Labels: Confirmed, Reported, Inferred, Conflicting, Unknown. Show event time, arrival time and feed-as-of.

HiLabs

Outreach and notes retain the same patient context

A failed contact is a recorded outcome. It is not a completed clinical assessment.



WALKTHROUGH 02

Riley's outreach

01 Suppress the known invalid phone route.

02 Record care-team coordination and a next review.

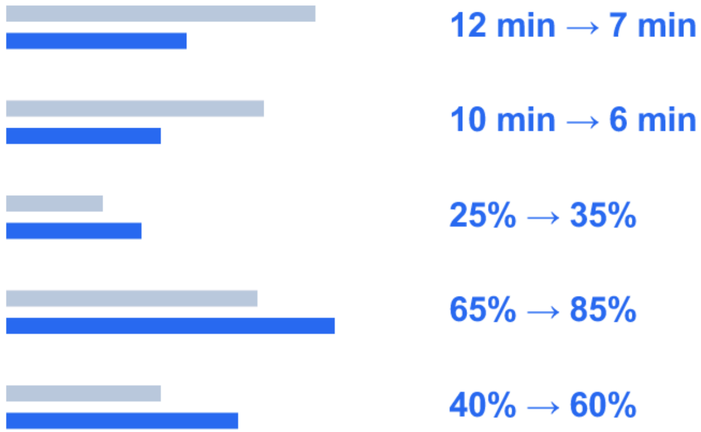
03 Draft a factual note without inventing patient assessment.

AI may assist with source-linked summaries and drafts. The demo uses a deterministic template. Humans confirm facts, outbound actions, care plans and signatures.

A narrow MVP has explicit measures and safeguards

Baselines and targets are pilot hypotheses. Replace assumptions with two weeks of measured baseline.

PILOT OUTCOME TARGETS



Preparation	**12 min → 7 min**
Documentation	**10 min → 6 min**
Outreach connect	**25% → 35%**
On-time follow-up	**65% → 85%**
Referral status verified	**40% → 60%**

Gray = assumed baseline Blue = pilot target

MVP now

Patient picture, explained queue, outreach, reviewed notes and owned follow-through.

Later

Arbitrary docking, live telephony, device feeds, new risk models and authorization integration.

Wrong or stale data

Preserve sources and dates. Confirm medication discrepancies.

Alert fatigue and bias

Deduplicate alerts. Explain ranking and review subgroup gaps.

Trust and context errors

Review drafts. Keep identity fixed and notes member-bound.

Hold caseload at 48 initially. Referral-status verification includes unavailable/declined outcomes; separately report actual service receipt. No clinical-effectiveness claim.

HiLabs

A first health-plan pilot tests workflow value

A working synthetic workbench supports the case. Clinical and integration validation comes before real use.

Buyer and sponsor

VP of Care Management, with nursing leadership as clinical sponsor.

Pilot scope

5–10 managers, roughly 200–400 eligible cases. Two baseline weeks, then six assisted-use weeks.

01 / BASELINE

2 weeks

Measure time and quality

02 / SHADOW

Initial assisted use

Verify sources and draft accuracy

03 / PILOT

6 assisted-use weeks

5–10 managers · 200–400 cases

Commercial hypothesis: fixed-fee pilot; then $200–$400 per active manager/month. Willingness-to-pay probe, not a market benchmark.

Live workbench

carethread-clinical-workbench.pranavbhatnagar17.chatgpt.site

Code and submission: github.com/pranav1785/care

Research register and limitations: Workbench_Specification.md. Primary sources: NCQA framework, AHRQ medication reconciliation, health-plan study (S2), CMS navigation evaluation.